

Consultation Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 7/3/2025 6:28 PM - 7:12 PM

Duration: 44 minutes

Service Code: 90834

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Note Content

Client reports seeking services to work through family conflict and differences in values and beliefs, and the mental, physical, and emotional impacts of those differences. Client reports "wanting support in reframing conversations" and ways to approach working with their family members as they have differing values and belief systems. Client reports experiencing "anxiety and depression" and currently being medicated for both but wanting support on how to manage those emotional experiences through talk therapy and other grounding and resourcing tools.

Client was in person at office for consultation.

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/14/2025 at 9:15 AM.

Consultation Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

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Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 7/3/2025 6:28 PM - 7:12 PM

Duration: 44 minutes

Service Code: 90834

Location: Main Office

Participants: Client Only

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 7/14/2025 at 4:47 PM.

Intake Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 7/9/2025 5:15 PM - 6:52 PM

Duration: 97 minutes

Service Code: 90791

Location: Main Office

Participants: Client only

Presenting Problem

Client reports "anxious ideation about interactions with my family in particular my parents that are distracting and intrusive." Client reports "I have to calm myself down from intrusive thoughts and anxious ideation after I finish dwelling on the intrusive thoughts and anxious ideation." Client reports "the stress and anxiety that comes from coming out to my family and the impacts of doing so during this time politically in American History."

Current Mental Status

Orientation:	X3: Oriented to Person, Place, and Time
General Appearance:	Appropriate
Dress:	Appropriate
Motor Activity:	Unremarkable
Interview Behavior:	Appropriate
Speech:	Normal
Mood:	Anxious
Affect:	Congruent
Insight:	Excellent
Judgment/Impulse Control:	Excellent
Memory:	Intact
Attention/Concentration:	Good
Thought Process:	Unremarkable
Thought Content:	Appropriate
Perception:	Unremarkable
Functional Status:	Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Objective Content

Therapist went over informed consent, policies, mandatory reporting, billing and cancellations.

Biopsychosocial Assessment

Identification:	Client reports being a white-American who is a 33 year old trans, non-binary, bi-sexual individual who is not religious and accepts atheist and agnostic labels. Client uses they/them pronouns.
History of Present Problem:	Client reports "symptoms go back to my childhood. I have always had a challenging relationship with my mom and our relationship is a source of anxiety for me." Client reports "avoidance of conflict brings on anxiety." Client reports experiencing "anxiety in cycles throughout the years and can range from daily to weekly." Client reports anxiety and depression symptoms last anywhere from "30 minutes to 2 hours."

Intake Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 7/9/2025 5:15 PM - 6:52 PM

Duration: 97 minutes

Service Code: 90791

Location: Main Office

Participants: Client only

Psychiatric History:

Client reports "I have received a general anxiety disorder diagnosis in early 2019 from a doctor and received anxiety medication as a result." Client reports "going to therapy in the past and started going in college in 2010-2011 and went regularly until 2016 with a break in 2015."

Trauma History:

Client reports relationship with their mom and being afraid of her emotional expression and her ways of being in conflict created avoidance habits and patterns. Client reports "fear of reprisal, harsh, and critical responses that created enough stress to want to avoid conflict." Client reports "these dynamics created social anxiety for myself." Client reports "moms emotional outburst felt unsafe." Client reports "relationship with father is avoidant of difficult conversations and the stress regarding my father is I don't know how to communicate difficult things with him. These dynamics bring obstacles to communication because of the stress that they bring on." Client reports "wider family are the most conservative in a fundamental way that causes complicated relationships with other members of my family. A lot of relational loss within family system due to conflicting views and beliefs."

Family Psychiatric History:

Client reports "my grandfather on mom's side was diagnosed with depression." Client reports "my mom received a diagnosis of depression this year." Client reports "all three siblings diagnosed with Generalized Anxiety Disorder. Younger brother is also diagnosed with Major Depressive Disorder, and younger sister's diagnosed with OCD. One sister also has neurological disorder including epilepsy symptoms."

Medical Conditions & History:

Client reports "childhood Asthma no longer taking medication for Asthma symptoms. As well as a gluten intolerance."

Allergies: Client reports "penicillin, seasonal allergies (9 months out of the year)."

Dental: Client reports "none."

Current Medications:

Client reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." Client reports ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." Allergy medication taken daily for allergies.

Substance Use:

Client reports "use alcohol and marijuana taken through edibles." Client reports "uses Alcohol 1-2 drinks 5-7x days a week. Edibles no more than a 10mg dose in a day, 3-5x a week."

Family History:

Client reports "both parents are still married, all birth siblings from parents, raised in CO." Client reports "relationship with parents is my mom and I want to be in each other lives but it is extremely hard for us to communicate. Relationship with my dad is much more friendly and we can carry on light conversations much more easily. Relationship with older of younger sister's least close with her out of all my siblings. We went through life and milestones simultaneously since she skipped a grade. She is competitive with me in a way I didn't wish to engage with. Less close with her as an adult. Younger sister of the two, wasn't close when we were younger but now closer with her as an adult. She is also queer. Sibling I am closest to. I am close with my younger brother as well but that happened more in adulthood as well."

Social History:

Client reports "I have a friend group here in Boulder not as close as past friend groups but am able to rely on them." Client reports "having close friendships that are long distance

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friendships." Client reports "struggle with romantic and intimate relationships and haven't had a partner in over a decade."

Spiritual/Cultural Factors: Client reports "none."

Developmental History: Client reports "none."

Educational/Vocational History: Client reports "finished undergrad, started a doctorate program and dropped out of it." Client reports "currently employed by Jefferson County Public Library as a data analyst." Client reports "hobbies and leisure activities include DND, queer board game get togethers, queer social events, solo/ and multiplayer video games, and reading."

Legal History: Client reports "none."

SNAP: Strengths: verbal communication and critical thinking skills, find it easy to not take myself too seriously, actively compassionate and show kindness towards people in moments of conflict

Growth edges: bad at defending myself in a conflict when the other person is less concerned about preserving the relationship, tend to struggle to appreciate how other people can't see multiple steps ahead in a conversation

Other Important Information: Client did not disclose perpetration of sexual or physical assault.

Client did not indicate challenges in ADLs: eating, bathing, getting dressed, toileting, mobility, and continence.

Client reports no barriers to treatment regarding cultural, power dynamics, or other dynamics within therapeutic relationship.

Client was in person at office for Intake session.

Plan

Therapist will create a treatment plan between this session and the next. At the beginning of the next session therapist will go over treatment plan with client.

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

- A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).
- B. The individual finds it difficult to control the worry.
- C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

- 1. Restlessness or feeling keyed up or on edge.
- 2. Being easily fatigued.
- 3. Difficulty concentrating or mind going blank.
- 4. Irritability.

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5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/11/2025 at 3:17 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 7/11/2025 at 4:48 PM.

Boulder Emotional Wellness

Date and Time: 7/23/2025 5:01 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

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Presenting Problem

Client reports "anxious ideation about interactions with my family in particular my parents that are distracting and intrusive." Client reports "I have to calm myself down from intrusive thoughts and anxious ideation after I finish dwelling on the intrusive thoughts and anxious ideation." Client reports "the stress and anxiety that comes from coming out to my family and the impacts of doing so during this time politically in American History."

Treatment Goal

Client would like to decrease negative and intrusive thoughts and increase capacity for discomfort that arises in interpersonal relationships when conflict is present. Client would like to decrease "anxiety" and increase internal resourcing and grounding.

Objective 1

Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills as reported in session.

Estimated Completion: 6 months (1/23/2026)

Boulder Emotional Wellness

Date and Time: 7/23/2025 5:01 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- EMDR grounding/resourcing exercises.
- Experiential Somatic Exercise.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Mindfulness Training.
- Nervous System Exercise.
- Nervous System Mapping.
- Parts work.
- Practice Nonviolent Communication.
- Preventative Services.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Symptom Management.

Objective 2

Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Estimated Completion: 6 months (1/23/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- EMDR grounding/resourcing exercises.

Boulder Emotional Wellness

Date and Time: 7/23/2025 5:01 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

-
- Experiential Somatic Exercise.
 - Exploration of Coping Patterns.
 - Exploration of Emotions.
 - Exploration of Nervous System Activation.
 - Exploration of Relationship Patterns.
 - Grounding/ resourcing.
 - Guided Imagery.
 - Interactive Feedback.
 - Interpersonal Resolutions.
 - Intrapersonal Resolutions.
 - Mindfulness Training.
 - Nervous System Exercise.
 - Parts work.
 - Open Ended Questioning.
 - Practice Nonviolent Communication.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 3

Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Estimated Completion: 6 months (1/23/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- EMDR grounding/resourcing exercises.
- Experiential Somatic Exercise.

Boulder Emotional Wellness

Date and Time: 7/23/2025 5:01 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

-
- Exploration of Coping Patterns.
 - Exploration of Emotions.
 - Exploration of Nervous System Activation.
 - Exploration of Relationship Patterns.
 - Grounding/ resourcing.
 - Guided Imagery.
 - Interactive Feedback.
 - Interpersonal Resolutions.
 - Intrapersonal Resolutions.
 - Mindfulness Training.
 - Nervous System Exercise.
 - Open Ended Questioning.
 - Parts work.
 - Practice Nonviolent Communication.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 4

There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Estimated Completion: 6 months (1/23/2026)

Prescribed Frequency of Treatment

Every 2 Weeks

I declare that these services are medically necessary and appropriate to the recipient's diagnosis and needs.

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/25/2025 at 2:05 PM.

Treatment Plan

Boulder Emotional Wellness

Date and Time: 7/23/2025 5:01 PM

Clinician: Nicole Fortunato, LPC

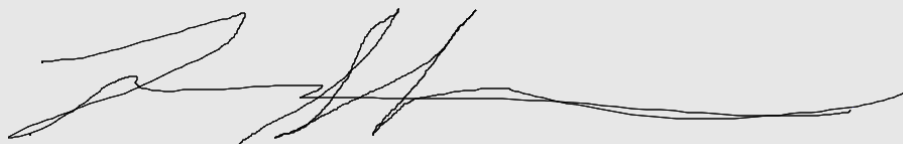
Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 7/25/2025 at 4:11 PM.

Signature

I acknowledge that I have participated in the development of the treatment plan and agree with the goals, objectives, and interventions.

A handwritten signature in black ink, appearing to be 'Tyler Shankel', written on a light gray background.

Tyler Shankel
9/12/2025

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 7/23/2025 5:17 PM - 6:23 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 7/23/2025 5:17 PM - 6:23 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." Client reports ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." Allergy medication taken daily for allergies.

Subjective Report and Symptom Description

Client reports "it makes me more anxious to work and communicate in the future" the impact of the power dynamics, how their feedback is received by leadership, and the future impact. Client reports "I am frustrated by the arrogance. It feel inescapable and there is lack of control" how client relates to one of the dynamics that shows up in the work dynamics. Client reports "I get stuck in dwelling and replaying comments" client's automatic coping strategies.

Objective Content

Therapist and client started by going over client's TP. Client resonated with goals and objectives offered and gave input into distress levels they are currently at. Therapist updated goals and objectives with client's distress level. Therapist then held space for client to check in. Client shared how they were feeling and a work dynamic that is causing them "stress." Therapist listened as client shared. Client spoke to how the power dynamics at work impact them and the interpersonal conflicts they find themselves in, that are reminiscent of personal relationships in their life. Therapist asked open ended questions, offered supportive reflections, emotional validation, reframes. Client also spoke to their coping strategies. Therapist offered psych-ed and ways to use grounding and resourcing tools when they are stressed. Client engaged with interventions used.

Interventions Used

The following interventions were used: Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Supportive Reflection, Review of Treatment Plan/Progress, Symptom Management, Interpersonal Resolutions, Psycho-Education, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

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3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at office for session.

Plan

Therapist and client will continue to explore client's interpersonal and intrapersonal relationship dynamics. Therapist will practice grounding and resourcing tools with client. Therapist will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for client to verbally process, working to decrease client's anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/28/2025 at 1:29 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 7/29/2025 at 5:15 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 8/5/2025 5:15 PM - 6:24 PM

Duration: 69 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

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D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

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Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 8/5/2025 5:15 PM - 6:24 PM

Duration: 69 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

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Medications

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Subjective Report and Symptom Description

Client reports feeling "increasingly frustrated that I have to accommodate" impact of relationship dynamics. Client reports feeling "lonelier and more isolated lately." Client reports "it is hard for me to not see it as consistently disappointing" a negative belief they hold around romantic relationships.

Objective Content

Therapist held space for client to check in. Client shared how they were feeling and spoke to multiple areas in their life that cause "anxiety." Therapist listened as client shared. Therapist and client explored the three areas of their life. Client spoke to the relationship dynamics within in their family system, in their friendships, and romantically. Therapist asked open ended questions and offered supportive reflections. Therapist and client explored client's "anxiety." Client named how their anxiety shows up internally and the impact of it externally on their relationships. Therapist offered emotional validation, psych-ed, reframes, and continued to ask open ended questions. Client engaged with interventions used.

Interventions Used

The following interventions were used: Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 8/5/2025 5:15 PM - 6:24 PM

Duration: 69 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at office for session.

Plan

Therapist and client will continue to explore client's interpersonal and intrapersonal relationship dynamics. Therapist will practice grounding and resourcing tools with client. Therapist will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for client to verbally process, working to decrease client's anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 8/11/2025 at 8:26 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 8/12/2025 at 4:52 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 8/20/2025 5:31 PM - 6:32 PM

Duration: 61 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 8/20/2025 5:31 PM - 6:32 PM

Duration: 61 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." Client reports ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." Allergy medication taken daily for allergies.

Subjective Report and Symptom Description

Client reports "I am having a hard time focusing and prioritizing productive tasks." Client reports "I quickly get overwhelmed by all the things to be considered." Client reports "I feel guilty for not being more present" in their relationship dynamics. Client has a belief if they were more present the relationship dynamic they spoke to would be "different."

Objective Content

Therapist held space for client to check in. Client shared how they were feeling and spoke to internal dynamics they are working with. Therapist listened as client shared. Therapist and client explored what is causing client's struggles around "focus and prioritizing productive tasks." Client stated feeling "overwhelmed" and is able to acknowledge where they get stuck but still struggles with moving forward. Therapist asked open ended questions, offered supportive reflections, and emotional validation. Therapist and client then explored client's relationship dynamics. Client shared an interaction that occurred in their friendship group and the beliefs they hold around the dynamics unfolding differently if they showed up differently. Therapist explored client's belief systems with them. As well as offered observations around a pattern of wanting to manage others emotional experiences. Client was open to therapist observations and shared the parts they identified with and the parts that didn't land. Therapist was open to client's corrections on their experience.

Interventions Used

The following interventions were used: Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 8/20/2025 5:31 PM - 6:32 PM

Duration: 61 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at office for session.

Plan

Therapist and client will continue to explore client's interpersonal and intrapersonal relationship dynamics. Therapist will practice grounding and resourcing tools with client. Therapist will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for client to verbally process, working to decrease client's anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 8/22/2025 at 9:13 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 8/25/2025 at 4:24 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 9/3/2025 5:15 PM - 6:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 9/3/2025 5:15 PM - 6:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." Client reports ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." Allergy medication taken daily for allergies.

Subjective Report and Symptom Description

Client reports "the fear is a spectrum. I fear their reaction being negative and either being hostile or withdrawn" clients fears connected to sharing their authentic self with their parents. Client reports feeling "contempt, disappointment, frustration, and very saddened" the emotional impact of their mom's behavior on them. Client reports "I would respect them less and it would shift how high of regard I hold them in" how client believes they would respond depending on their parents reaction.

Objective Content

Therapist held space for client to check in. Client shared how they were doing and also spoke to relationship dynamics with their parents. Therapist listened as client shared. Therapist and client explored clients relationship dynamics with their mom. Client spoke to their moms rigidity and conflicting beliefs, values, and views. Client shared how they relate to their mom and what their hopes, desires, and fears for how their relationship looks. Therapist asked open-ended questions, offered supportive reflections, and emotional validation. Therapist inquired with client about how they feel in regards to their relationship dynamics with their mom. Client initially spoke to the beliefs and thoughts that they hold about their mom's actions. Therapist inquired again about client's emotional experience. Client then spoke to the emotional impact of the dynamics. Client expressed their "frustrations, anxieties, and disappointment." Therapist used parts work, offered psycho education and reframes. Client engaged with interventions used and stated feeling "a bit lighter and that it was good to talk about" by the end of session.

Interventions Used

The following interventions were used: Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 9/3/2025 5:15 PM - 6:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at office for session.

Plan

Therapist and client will continue to explore client's interpersonal and intrapersonal relationship dynamics. Therapist will practice grounding and resourcing tools with client. Therapist will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for client to verbally process, working to decrease client's anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/4/2025 at 8:06 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 9/5/2025 at 8:17 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 9/17/2025 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 9/17/2025 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." Client reports ".1mg of Estradiol administered by patch twice a week, estrogen for HRT." Allergy medication taken daily for allergies.

Subjective Report and Symptom Description

Client reports "I've been worrying and having anxiety around the dynamics with my friend" client speaking to the emotional impact of the relationship dynamics with their friend. Client reports "I had social dysphoria and I experienced sadness, depression, anxiety, and sorrow, and I couldn't pinpoint why I felt that way" client speaking to the emotional impact of their major life transition. Client reports "I had trauma from being unemployed for 18 months that I took whatever job was available and now I have guilt for having worked in a place that is the opposite of my beliefs and values" client sharing the impact of the choices they made.

Objective Content

Therapist held space for client to check in. Client shared updates on how they were feeling, as well as, interpersonal relationship dynamics with their friend. Therapist listened as client shared, offering supportive reflections and asking open-ended questions. Therapist and client then moved into exploring client's process around supporting client through a major life transition. Client shared the different aspects of their major life transition including the past, present, and future components of the transition. Client spoke to the emotional impact the different phases and areas their major life transition has and continues to have on them. Therapist continued to ask open-ended questions and offer supportive reflections. Therapist also offered emotional validation, psych-ed, and explored the different parts of themselves using Parts work. Client also shared how previous choices that they've made bring up "guilt due to those choices being out of alignment with their values and beliefs." Therapist continued to offer emotional validation and hold space for client to share the emotional impact of the choices they've made. Client engaged with interventions used and stated "it was helpful and I'm grateful for revisiting the old stuff."

Interventions Used

The following interventions were used: Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 9/17/2025 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at office for session.

Plan

Therapist and client will continue to explore client's interpersonal and intrapersonal relationship dynamics. Therapist will practice grounding and resourcing tools with client. Therapist will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for client to verbally process, working to decrease client's anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/29/2025 at 4:24 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 9/30/2025 at 3:20 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/1/2025 5:16 PM - 6:16 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/1/2025 5:16 PM - 6:16 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." clt. reports taking ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." clt. reports "allergy medication is taken daily for allergies."

Subjective Report and Symptom Description

clt. reports "I was most uncomfortable with the disparities of class and how put together everyone was and the lack of trauma those with money seem to have" client sharing how they related to those with more economic wealth and the way they perceived those they were interacting with.

clt. reports "I had anxious ideations, I was having emotionally self-harming thoughts, and it has been easier to turn them off with SSRI's"

clt. sharing what they noticed has been helpful for their anxious thoughts and ideations.

clt. reports "I needed to talk bluntly about it" clt. sharing why the support in therapy is good for them.

Objective Content

th. and clt. explored how recent friend interaction went. clt. named the impact the interaction had on how they were viewed and the impact others perceptions has on them. th. asked open ended questions, offered active listening, emotional validation, and supportive reflections as clt. spoke. th. and clt. explored the relationship dynamics that showed up in interaction. clt. spoke to financial disparities and how their history impacted how they engaged and related to others in a different economic class. clt. expressed feeling "anxious and envious" regarding the dynamics. th. explored clt's. emotions with them. clt. struggles to name their emotional experience and instead goes into story versus acknowledging the actual feeling. th. is working with clt. on building their capacity and vocabulary for emotions. clt. shared what has been helpful to manage their emotional experience. th. offered psych-ed, reframes, grounding and resourcing tools, continued to ask open ended questions, and offer emotional validation and supportive reflections. clt. engaged with intervention used throughout session.

Interventions Used

The following interventions were used: Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/1/2025 5:16 PM - 6:16 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was at home for telehealth session.

Plan

th. and clt. will continue to explore clt's. interpersonal and intrapersonal relationship dynamics. th. will practice grounding and resourcing tools with clt. th. will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for clt. to verbally process, working to decrease clt's. anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 10/5/2025 at 8:06 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 10/7/2025 at 3:42 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/15/2025 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/15/2025 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." clt. reports taking ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." clt. reports "allergy medication is taken daily for allergies."

Subjective Report and Symptom Description

clt. reports "I had a hard week. I was upset and was not feeling prioritized" clt. sharing how they were feeling and speaking to the impact relationship dynamics are having on them.

clt. reports "I feel lonely, sad, and disappointed" clt. sharing their emotional experience and the emotions clt. was able to name and connect with.

clt. reports "there is a fear of judgements from others" clt. sharing one of their fears that arises in interpersonal relationship dynamics.

Objective Content

clt. started by sharing how they were feeling and spoke to the impact relationship dynamics are having on them. th. listened as clt. shared, tracking themes and patterns. th. asked clt. what they noticed about how they were feeling. clt. struggles to acknowledge the feeling without going into story about how they relate. th. is tracking this pattern in clt. th. and clt. explored clt's. relationship dynamics. clt. named struggling to connect in hobbies and activities with others because they relate to social interactions differently than their counterparts. th. offered supportive reflections, emotional validation, and asked open ended questions. th. and clt. explored the theme of "pressure and expectations" that show up in their relationship dynamics. clt. was able to name and acknowledge the pressure and expectations they have of others and themselves when it comes to setting up the interaction. th. offered reframes and psych-ed. clt. engaged with interventions used throughout session and expressed "appreciation for having space to share and explore their experience."

Interventions Used

The following interventions were used: Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/15/2025 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in person at office for therapy session.

Plan

th. and clt. will continue to explore clt's. interpersonal and intrapersonal relationship dynamics. th. will practice grounding and resourcing tools with clt. th. will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for clt. to verbally process, working to decrease clt's. anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 10/19/2025 at 2:23 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 10/20/2025 at 6:22 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/29/2025 5:18 PM - 6:16 PM

Duration: 58 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/29/2025 5:18 PM - 6:16 PM

Duration: 58 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." clt. reports taking ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." clt. reports "allergy medication is taken daily for allergies."

Subjective Report and Symptom Description

clt. reports "I was waiting for it to feel right" clt. sharing what is stopping them following through on their decision connected to their life transition.

clt. reports "it's quite frustrating" clt. speaking to the relationship dynamics connected to their decision.

clt. reports "my reaction is hesitancy and skepticism" clt. speaking to their response to the relationship dynamics they were speaking to.

Objective Content

clt. started by sharing how they were feeling and then brought back in a previous topic connected to an upcoming decision they are continuing to navigate around a life transition. th. listened as clt. shared. th. and clt. explored clt's. fears and relationship dynamics connected to their decision. clt. shared their fears and the "uncertainty that comes with not knowing how the other people in the decision are going to respond." clt. stated "there is a fear of loss of relationship and isolation." th. asked open ended questions, offered supportive reflections, and emotional validation. th. and clt. also explored clt's. emotions. clt. stated "it will hurt if there reaction is significantly negative." th. notices clt. struggles to emote but can name their emotions and speaks to and about their emotions from a logical perspective. clt. engaged with interventions used throughout session and expressed appreciation for th. support and space to explore the topic they shared in session.

Interventions Used

The following interventions were used: Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 10/29/2025 5:18 PM - 6:16 PM

Duration: 58 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in person at office for therapy session.

Plan

th. and clt. will continue to explore clt's. interpersonal and intrapersonal relationship dynamics. th. will practice grounding and resourcing tools with clt. th. will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for clt. to verbally process, working to decrease clt's. anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 11/3/2025 at 7:58 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 11/4/2025 at 1:00 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 11/12/2025 5:15 PM - 6:15 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 11/12/2025 5:15 PM - 6:15 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." clt. reports taking ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." clt. reports "allergy medication is taken daily for allergies."

Subjective Report and Symptom Description

clt. reports "there is a lot of stress and hurt feelings" clt. sharing the emotional impact of the work dynamics.

clt. reports "I'm not a hot anger person, I am a cold anger person meaning I get more serious and less compromising than expressive" clt. sharing more about his relationship to anger.

clt. relates to his anger and works with his anger in the same way he does in his family system where he doesn't engage and add fuel to the fire which is why he goes more inwards and becomes more boundary.

Objective Content

clt. started by sharing how he was feeling and spoke to work stressors. th. listened as clt. shared. th. and clt. explored clt's. work stressors. clt. shared the dynamics happening between his team and upper management. clt. shared the impacts of the dynamics between both parties and stated "they are trying to escape accountability which is frustrating." th. asked open ended questions, offered supportive reflections, and emotional validation. th. and clt. explored clt's. emotions and focused on his relationship to anger. clt. shared there is "friction" in the dynamics and stated "showy anger makes me uncomfortable" and then spoke to how anger shows up in his system. th. continued to offer emotional validation, ask open ended questions, and offered an observation regarding clt's. relationship to anger. clt. expressed resonance with th. observation and stated "it is helpful to have space to be blunt" talking about his experience in a way he can't be in other relationships and contexts.

Interventions Used

The following interventions were used: Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Interpersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 11/12/2025 5:15 PM - 6:15 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was at home for telehealth session.

Plan

th. and clt. will continue to explore clt's. interpersonal and intrapersonal relationship dynamics. th. will practice grounding and resourcing tools with clt. th. will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for clt. to verbally process, working to decrease clt's. anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 11/12/2025 at 6:32 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 11/14/2025 at 5:26 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 11/26/2025 5:15 PM - 6:18 PM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Tangential

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 11/26/2025 5:15 PM - 6:18 PM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." clt. reports taking ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." clt. reports "allergy medication is taken daily for allergies."

Subjective Report and Symptom Description

clt. reports "I would like to skill to relate proportionally to mistakes."

clt. reports "I am extremely careful and thoughtful about choices. I have a huge fear of making a mistake. I want to avoid the worry and mystery of being able to make a mistake."

clt. reports "my immediate reaction is always to worry. I can't find a balance, there's no middle space."

Objective Content

clt. started by sharing how he was doing and stated "being disproportionately stressed this week from a work stressor." clt. shared that this brought up curiosities around being diagnosed with OCD. th. listened as clt. shared. th. and clt. explored clt's. symptoms and responses connected to being disproportionately stressed. clt. stated "small things cause big stressors and I can't isolate from them. It's either all or nothing." clt. also stated "it takes a long time to make a decision. I have to thoroughly investigate all of the choices." th. asked open-ended questions, offered supportive reflections, and emotional validation. th. and clt. explored clt. receiving an OCD diagnosis. th. notices clt's. persistent need to dissect his choices to manage his heightened anxiety connected to the need for control and perfectionism. clt. is able to acknowledge how his OCD behaviors show up. clt. stated "I get frustrated at not being able to ignore imperfection." clt. also stated "I get hyper fixated on the choices I make. I have a low tolerance for imperfection, and I become very stressed and anxious trying to avoid making mistakes." clt. states these are daily and weekly occurrences and they get in the way of him having proportionate responses to external stressors. th. offered psych-ed and will continue to look into offering the appropriate diagnosis for client based on his symptoms and level of distress connected to his symptoms.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 11/26/2025 5:15 PM - 6:18 PM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in person at office for therapy session.

clt. has persistent anxiety and OCD behaviors. clt. needs continued therapeutic support to work with decreasing clt's. anxiety and offering a proper diagnosis for clt's. OCD symptoms. clt. hadn't shared much about his OCD behaviors and clt. presents with dual diagnosis which had been taking precedent in session from th. observation.

Plan

th. and clt. will continue to explore clt's. interpersonal and intrapersonal relationship dynamics. th. will practice grounding and resourcing tools with clt. th. will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for clt. to verbally process, working to decrease clt's. anxiety. th. and clt. will continue to explore an appropriate and accurate diagnosis for clt. to receive the proper care support he needs to decrease and manage his symptoms that lead to increased and disproportionate distress.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/7/2025 at 9:04 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 12/9/2025 at 1:41 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 12/10/2025 5:15 PM - 6:14 PM

Duration: 59 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 12/10/2025 5:15 PM - 6:14 PM

Duration: 59 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." clt. reports taking ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." clt. reports "allergy medication is taken daily for allergies."

Subjective Report and Symptom Description

clt. reports "I deprioritize things to not have to worry about making them happen" clt. speaking to how he manages his OCD behaviors and impulses.

clt. reports "I feel down about feeling down" clt. sharing how his self-aggression shows up in relation to negative emotions.

clt. reports avoiding "rejection and emotional distress" speaking to how he will disengage from experiences or tasks that illicit these emotional experiences to not feel the negativity or negative stories and beliefs connected to these emotions.

Objective Content

th. started by revisiting clt's. request and inquiry regarding an OCD diagnosis. th. provided updated information with clt. th. is continuing to seek supervision support around offering OCD as a diagnosis. clt. expressed understanding for information th. provided. th. and clt. then moved into exploring clt's. relationship to his emotions and experiences that bring up "sadness, disappointment, and anxiety." clt. was able to allow himself a little bit of space to feel his emotions but does keep his emotions reserved and tends towards logic instead of expression. th. is tracking clt's. relationship to his emotions and is offering gentle perspectives shifts, emotional validation, supportive reflections, and asked open ended questions. th. and clt. explored how clt. relates to a predominately negative experience he is hoping to shift his relationship to. clt. expresses "fear of past experiences repeating themselves" and goes into self-criticism as to why the experiences keep unfolding negatively. th. offered psych-ed, reframes, and ways to work with grounding and resourcing his system when negative beliefs and emotions arise. clt. expressed appreciation for th. validation and reflections, and expressed understanding and interest in implementing grounding and resourcing exercises offered.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 12/10/2025 5:15 PM - 6:14 PM

Duration: 59 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in person at office for therapy session.

clt. has persistent anxiety and OCD behaviors. clt. needs continued therapeutic support to work with decreasing clt's. anxiety and offering a proper diagnosis for clt's. OCD symptoms. clt. hadn't shared much about his OCD behaviors and clt. presents with dual diagnosis which had been taking precedent in session from th. observation.

th. is still working on providing the appropriate diagnosis for clt.

Plan

th. and clt. will continue to explore clt's. interpersonal and intrapersonal relationship dynamics. th. will practice grounding and resourcing tools with clt. th. will offer PRI, Parts Work, Polyvagal Theory, somatic exercises, and hold space for clt. to verbally process, working to decrease clt's. anxiety. th. and clt. will continue to explore an appropriate and accurate diagnosis for clt. to receive the proper care support he needs to decrease and manage his symptoms that lead to increased and disproportionate distress.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/18/2025 at 11:52 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 12/18/2025 at 1:31 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 1/7/2026 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 1/7/2026 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." Clt. reports taking ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." Clt. reports "allergy medication is taken daily for allergies."

Subjective Report and Symptom Description

Clt. reports "there is fear of a negative reaction" which is leading clt. to try to figure out how to share information that will be received without the other person in the dynamic having a reaction. Clt. has a high need to manage his and others emotional experience to not experience loss and heightened emotional distress.

Clt. reports "I become an anxious mess and unproductive" clt. sharing why he is trying to manage his and others emotional response.

Clt. reports "I am trying to protect the relationship" clt's. motivation as to why he manages emotions in self and others and why he becomes more anxious around needing to get the planning of how and what he is going to say right.

Objective Content

Th. and clt. explored clt's. anxiety. Clt. shared relationship dynamics that are leading to heightened states of anxiety with symptoms including "intrusive thoughts, excessive worry, avoidance of external reminders, and heightened emotional distress" connected to fears of rejection and loss of relationship. Clt. stated a need to "manage my and their emotions" to avoid the loss of relationship and the heightened emotional distress. Clt. has a low tolerance and capacity to be with emotional distress connected to loss. Th. and clt. explored clt's. coping strategies connected to loss and emotional distress. Clt's. automatic response is to avoid the stressors and emotional pain. Throughout session th. provided psych-ed, emotional validation, supportive reflections, grounding and resourcing and non-violent communication tools and resources, and asked open ended questions to support clt. in his process and decrease his anxiety and distress. Clt. engaged with interventions used and expressed understanding and resonance for psych-ed and tools offered.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 1/7/2026 5:15 PM - 6:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was in person at office for therapy session.

Clt. has persistent anxiety and OCD behaviors. Clt. needs continued therapeutic support to work with decreasing clt's. anxiety and increasing clt's. knowledge and application of tools and resources.

Trauma-focused treatment protocols required additional time to complete the interventions without destabilizing the clt.

Plan

Th. and clt. will go over and update clt's. TP. Th. will administer an OCD evaluation with clt. to determine if OCD is the appropriate diagnosis or if Generalized Anxiety Disorder his current diagnosis is the appropriate diagnosis.

Recommendation: Change treatment goals or objectives

Prescribed Frequency of Treatment: Every 2 Weeks

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 1/10/2026 at 6:42 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 1/12/2026 at 6:46 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 1/21/2026 5:15 PM - 6:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 1/21/2026 5:15 PM - 6:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "20 mg of Citalopram brand name Cilexia for anxiety and depression taken daily." Clt. reports taking ".1mg of Estriodol administered by patch twice a week, estrogen for HRT." Clt. reports "allergy medication is taken daily for allergies."

Subjective Report and Symptom Description

Clt. reports "social situations aren't as predictable which heightens my anxiety and there is a need for perfectionism" which is where his OCD compulsions come into play. Clt's. OCD compulsions are his coping patterns for regulating his anxiety.

Clt. reports "I fill ambiguity with worst case scenarios" which increases his anxiety.

Clt. reports "I would like to find ways of motivating myself that aren't so harmful. Right now I am threatening myself with judgement."

Objective Content

Th. administered OCD questionnaire to explore severity of clt's. OCD symptoms. The questionnaire took the majority of the session to complete. During the questionnaire clt. stated "it is hard to talk myself out of negative belief systems based on how I feel. I start scanning my memories for what went wrong and play out the whole thought chain before I can move on." Clt. gets stuck in his self-aggressive intrusive thoughts and tries to decrease his anxiety using compulsive behaviors to regulate his NS. Clt. qualifies for both GAD and OCD diagnoses. Th. will plan to complete clt's. TP in the next session using the GAD diagnosis.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 1/21/2026 5:15 PM - 6:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was in person at office for therapy session.

Clt. has persistent anxiety and OCD behaviors. Clt. needs continued therapeutic support to work with decreasing clt's. anxiety and increasing clt's. knowledge and application of tools and resources.

Trauma-focused treatment protocols required additional time to complete the interventions without destabilizing the clt.

Plan

Th. and clt. will go over and update clt's. TP since th. ran out of time due to OCD questionnaire taking the majority of the session.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 2/5/2026 at 6:15 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 2/7/2026 at 10:23 AM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 2/4/2026 5:15 PM - 6:22 PM

Duration: 67 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 2/4/2026 5:15 PM - 6:22 PM

Duration: 67 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Subjective Report and Symptom Description

Clt. reports "my anxieties and worries are becoming more stressful and impacting my actions and decisions."

Clt. reports "I implement distraction to cope and I need to get it right because if I'm not going to do it well, I'm not going to do it at all" how he resources his anxiety and also how his anxiety shows up.

Clt. reports "I avoid the external stimulus to avoid the negative internal reaction."

Objective Content

Th. and clt. started by going over and updating clt's. TP plan to reflect clt's. progress and areas the clt. continues to struggle in. Based on the clt's. self report and th. observations, th. is going to keep clt. under the GAD diagnosis as clt. acknowledged using compulsive behaviors to regulate his and anxiety and NS dysregulation. Clt. has overlapping symptoms with GAD and OCD and is wanting to focus his attention on his GAD symptoms. Th. will reevaluate clt's. diagnosis in 6 months at the next treatment plan reevaluation to decide if th. and clt. will continue current therapeutic focus or switch to focusing on clt's. OCD symptoms. Going over and updating clt's. TP took the majority of the session. Th. and clt. used the remainder of the time to discuss clt's. "anxiety, worry, and intrusive thoughts." Clt. shared an area of his life that his anxiety, worry, and intrusive thoughts become heightened and he struggles to currently resource himself out his anxiety and intrusive thoughts. Th. asked open ended questions, offered supportive reflections, emotional validation, reframes, and psych-ed. Clt. engaged with interventions used. Th. gave clt. HW to track and reflect on where the messaging he received from his anxious intrusive thoughts come from. Clt. expressed willingness to track HW offered and report back what he notices.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Review of Treatment Plan/Progress, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will identify and implement self-soothing techniques to use at least 3x a week to decrease "anxiety" and increase emotional regulation skills aeb client report in session.

Progress: Progressing

2. Client will increase their tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring client back into relationship with their body and rewire their relationship to their anxiety. Client will decrease their discomfort from a level 7 to 6 in the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC
Supervisor: Erica Johnson, LPC
Patient: Tyler Shankel, DOB 8/6/1991

Date and Time: 2/4/2026 5:15 PM - 6:22 PM

Duration: 67 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

3. Client will work on decreasing the negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle their negative belief systems and shift their perspective. Client will achieve this by implementing compassionate beliefs and reframes when they notices they are talking negatively to themselves. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Progress: Progressing

4. There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was in person at office for therapy session.

Clt. has persistent anxiety and OCD behaviors. Clt. needs continued therapeutic support to work with decreasing clt's. anxiety and increasing clt's. knowledge and application of tools and resources.

Trauma-focused treatment protocols required additional time to complete the interventions without destabilizing the clt.

Plan

Th. and clt. will continue to explore and work with decreasing and concurrent create capacity for clt. to be with his anxiety, anxious intrusive thoughts, and coping patterns. Th. will practice grounding and resourcing tools with clt. Th. will offer PRI, IFS, Polyvagal Theory, somatic and validation exercises, and hold space for clt. to verbally process, working to decrease clt's. anxiety.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 2/5/2026 at 6:32 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 2/7/2026 at 10:24 AM.

Boulder Emotional Wellness

Date and Time: 2/4/2026 5:18 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Clt. reports experiencing "excessive anxiety and worry (apprehensive expectation) at work but mainly in social interactions," finds it difficult to control the worry: ruminating on, turning over, catastrophizing, and imaging worst case scenarios," feeling fatigued, restlessness and feeling keyed up or on edge, difficult concentrating and being distracted by anxious intrusive thoughts, the anxiety and worry are demotivating and struggle to take actionable steps to do other productive or related tasks, can't progress major things I want to accomplish because I get stuck in the worry and anxiety.

Clt. has OCD compulsions and behaviors connected to social worry and anxiety. Currently GAD is the most appropriate diagnosis and and OCD diagnosis will continue to be evaluated for future shifts in diagnosis.

Presenting Problem

Client reports "anxious ideation about interactions with my family in particular my parents that are distracting and intrusive." Client reports "I have to calm myself down from intrusive thoughts and anxious ideation after I finish dwelling on the intrusive thoughts and anxious ideation." Client reports "the stress and anxiety that comes from having important and emotional conversations with my family and the impacts of doing so during this time politically in American History."

Treatment Goal

Boulder Emotional Wellness

Date and Time: 2/4/2026 5:18 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Tyler Shankel, DOB 8/6/1991

Client would like to decrease the debilitation and energy consumption of negative intrusive thoughts and increase the management and resources applied when the negative intrusive thoughts arise. Client would like to build capacity for discomfort that arises in interpersonal relationships when conflict is present. Client would like to decrease "anxiety" and increase internal resourcing and grounding.

Objective 1

Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease symptoms of "anxiety, worry, and negative intrusive thoughts" and increase NS regulation. Client will decrease his distress connected to his generalized "anxiety, worry, and negative intrusive thoughts" levels from a level 7 to 5 a level; and when interfacing with family will decrease his "anxiety, worry, and negative intrusive thoughts" from a level 9 to a level 7, over the next six months aeb client report in session.

Estimated Completion: 6 months (8/4/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Mindfulness Training.
- Nervous System Exercise.
- Nervous System Mapping.
- Practice Nonviolent Communication.
- Preventative Services.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Symptom Management.

Objective 2

Client will increase his tolerance for the discomfort that arises when "conflict" is present. Client will achieve this by using somatic based exercises that bring the client back into relationship with his body that will also shift and rewire how he relates to his anxiety,

Boulder Emotional Wellness

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worry, and negative intrusive thoughts somatically. Client will decrease his physical discomfort from a level 6 to a level 5, over the next six months aeb client report in session.

Estimated Completion: 6 months (8/4/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Interpersonal Resolutions.
- Intrapersonal Resolutions.
- Mindfulness Training.
- Nervous System Exercise.
- Open Ended Questioning.
- Practice Nonviolent Communication.
- Preventative Services.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Supportive Reflection.
- Symptom Management.

Objective 3

Client will work on decreasing his negative beliefs and stories connected to conflict in interpersonal relationships by using reframes and cognitive challenging to dismantle his negative belief systems and shift his perspective. Client will achieve this by implementing compassionate beliefs and reframes when he notices he is talking negatively to himself. Client will implement the practice of reframing and cognitive challenging at least 3x a week aeb client report in session.

Estimated Completion: 6 months (8/4/2026)

Treatment Strategy / Intervention

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-
- Cognitive Challenging.
 - Cognitive Refocusing.
 - Cognitive Reframing.
 - Communication Skills.
 - EMDR grounding/resourcing exercises.
 - Exploration of Coping Patterns.
 - Exploration of Emotions.
 - Exploration of Nervous System Activation.
 - Exploration of Relationship Patterns.
 - Grounding/ resourcing.
 - Guided Imagery.
 - Interactive Feedback.
 - Interpersonal Resolutions.
 - Intrapersonal Resolutions.
 - Mindfulness Training.
 - Nervous System Exercise.
 - Open Ended Questioning.
 - Practice Nonviolent Communication.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 4

There are currently no barriers to treatment regarding cultural, power, or other dynamics in therapeutic relationship and client report in session.

Estimated Completion: 6 months (8/4/2026)

Additional Information

Client reports having increased his knowledge and understanding of psych-ed, tools and resources, however, client noted still experiencing heightened levels of "anxiety, worry, and negative intrusive thoughts"; continued therapy is needed to help client continue to work towards his goal of lowering his symptoms of "anxiety, worry, and negative intrusive thoughts" and increase his knowledge and application of tools and resources.

Treatment Plan

Boulder Emotional Wellness

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Prescribed Frequency of Treatment

Every 2 Weeks

I declare that these services are medically necessary and appropriate to the recipient's diagnosis and needs.

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 2/5/2026 at 6:17 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License CO 14267, signed this note and declared this information to be accurate and complete on 2/7/2026 at 12:17 PM.

Signature

I acknowledge that I have participated in the development of the treatment plan and agree with the goals, objectives, and interventions.

A handwritten signature in black ink, appearing to be 'Tyler Shankel', written over a light gray background.

Tyler Shankel
2/12/2026